

Dissertation Article Source Notes Packet

Organized by article for Destynnee’s dissertation: staff-facing rehabilitation manual/program for incarcerated or justice-involved emerging adult men using positive psychology, basic CBT, trauma-informed practice, correctional feasibility, staff training/fidelity, RNR/desistance, and emerging adulthood.

Source files analyzed	5 uploaded documents: 4 PDFs + 1 DOCX manuscript
Highlight status	No PDF annotation-layer highlights detected. Notes are direct article notes and dissertation-relevant
Organization	Each article is separated with citation, purpose, sample/design, data points, findings, quotes, limitations

Articles Included

- 1) Vaswani & Paul (2019) — “It’s knowing the right things to say and do”: challenges and opportunities for trauma-informed practice in the prison context
- 2) Levenson & Willis (2018) — Implementing Trauma-Informed Care in Correctional Treatment and Supervision
- 3) Dumornay et al. (2022) — Improved emotion regulation following a trauma-informed CBT-based intervention associates with reduced risk for recidivism in justice-involved emerging adults
- 4) Taxman (2014) — Building effective service delivery mechanisms for justice-involved individuals: an under-researched area
- 5) Klapow, Shea, Bohn, & Hoffman — The Effectiveness of Positive Psychology-Based Interventions in Prisons on Well-Being: a Systematic Review and Meta-Analysis

Source-Organized Dissertation Article Notes

Prepared for Destyne's dissertation packet. Topic focus: staff-facing rehabilitation manual/program for incarcerated or justice-involved emerging adult men using positive psychology, basic CBT, trauma-informed practice, correctional feasibility, staff training/fidelity, and RNR/desistance/emerging-adulthood framing.

1) Vaswani & Paul (2019) — “It’s knowing the right things to say and do”: challenges and opportunities for trauma-informed practice in the prison context

Citation / identification

- Vaswani, N., & Paul, S. (2019). “It’s knowing the right things to say and do’: Challenges and opportunities for trauma-informed practice in the prison context.” *The Howard Journal of Crime and Justice*.
<https://doi.org/10.1111/hojo.12344>
- Source file: doc_fd7242878ff5_Trauma informed staff.txt
- Stable page markers: Yes, extraction includes PAGE 1-27 markers.

Purpose

- Explores prison staff perspectives on implementing trauma-informed practice (TIP) in a young offender institution.
- Focuses on the gap between policy/vision and real-world prison practice.
 - Argues that prisons may become more “trauma-aware,” but truly trauma-informed practice is constrained by the punitive nature of custody.

Sample / population

- Over 200 prison staff in a Scottish Young Offenders’ Institution (YOI).
- Questionnaire data from 208 staff attending trauma, bereavement, and loss awareness training.
- Staff roles included all levels/roles, but most were uniformed prison staff: Residential Officers and Personal Officers, n = 129, 62%.
- Additional qualitative data: 3 focus groups with 12 staff and 10 semi-structured interviews with managers/service providers.

Method / design

- Secondary qualitative analysis of data from an evaluation of an establishment-wide trauma-informed shift.
- Data sources: pre/post training questionnaires, focus groups, interviews.
- Thematic analysis using NVivo 10.
- Six themes: roles/responsibilities/relationships; shared experiences/empathy/understanding; organizational change; needs/resources; prison environment; professional support/supervision.

Important facts / data points / statistics

- Prison populations show elevated loss, trauma, victimization, abuse, mental health needs, traumatic head injuries, childhood bereavement, and ACEs (pp. 2-5).
- Environmental triggers in prison include crowding, bright lights, inescapable noise, solitary confinement, fear, violence, and substance withdrawal (p. 4).

- Scottish serious prisoner-on-prisoner assaults: 13 per 1,000 prisoners in 2017/18, compared to 36 per 1,000 in England/Wales (p. 4).
- Young people in custody are developmentally vulnerable; under-20s are at increased risk of self-harm in prison (p. 5).
- TIP principles identified: trust, collaboration, choice, empowerment, and safety (p. 5).
- Training intervention: mandatory two-day trauma, bereavement, and loss awareness training for all YOI staff. Topics included bereavement models, trauma's developmental impact, trauma symptoms/coping, PTSD, attachment/relationships, culture/systems/practice, safe working practices, and role-play (p. 11).

Key findings

- Staff often already performed trauma-consistent practices: listening, emotional support, practical problem-solving, noticing behavioral changes, encouraging family contact (pp. 12-13).
- Training helped legitimize and label work that staff were already doing, increasing confidence and willingness to ask difficult questions or make referrals (pp. 13-14).
- Organizational culture showed small positive shifts: language changes, more knowledge of trauma/bereavement/loss, reduced shame/stigma, peer word-of-mouth referrals (pp. 14-15).
 - *Major barriers: overwhelming needs, limited staff time, waiting lists, inadequate community supports, restricted regime, lack of privacy, limited therapeutic spaces, hypermasculine culture, "them and us" staff-prisoner dynamics, care/control tension, and insufficient staff supervision (pp. 15-20).*
- Frontline prison staff may experience emotional labor, vicarious trauma, compassion fatigue, burnout, and detachment/depersonalization, but lack the supervision culture common in health/social care professions (pp. 7-8, 19-22).
- Authors conclude that prison's punitive purpose, power imbalances, fear, mistrust, restricted regimes, and negative staff attitudes prevent full TIP implementation (pp. 21-24).

Useful direct quotes / near-quotes

- *"Responding to trauma is everybody's business" (quoted from NHS Education for Scotland framework, p. 2).*
- *TIP is at minimum a "do no harm" approach (p. 2).*
- *"It is not possible for a custodial establishment to become truly trauma-informed while punishment remains a fundamental element of the prison system" (near-quote from argument, p. 3).*
- *Staff role/practice quote: "Work with them on a daily basis, get to know them and understand what is 'normal' behaviour for them, so when something isn't normal I can pick up on this. I make time, take an interest. I speak with them." (Residential Officer, p. 13).*
- *Training effect quote: "Probably before that course I wouldn't have known how to handle it or what to say to him..." (Instructor/Tutor/Activities, p. 14).*
- *Time/resource barrier: "Spending the necessary time with individuals during loss can be difficult to manage as you could have as many as 40 other young people in your care." (Residential Officer, p. 16).*
- *Privacy barrier: "Difficult to offer support behind a closed door. Difficult to talk to them especially when other young people can hear what is said." (Operations Officer, p. 17).*
- *Staff skill/confidence quote: "It's knowing the right things to say and do when dealing with people affected by trauma, bereavement and loss." (Residential Officer, p. 19).*
- *Supervision barrier: "...you're scared because we don't have supervision at the back of us so although you want to ask the question you don't because you're thinking where will this lead to..." (Instructor/Tutor/Activities, p. 20).*
- *Conclusion: "TIP is not possible without significantly reshaping the current penal system" (near-quote, p. 24).*

Limitations / cautions

- TIP was not the original explicit focus of the broader evaluation; this paper reanalyzed data from staff perspectives.
- Focused on young males in custody in a Scottish YOI; findings may differ for adult men, women, U.S. prisons/jails, or community supervision.
- Qualitative design supports depth/context, not causal estimates.

Dissertation relevance

- Strong support for staff-facing manual sections explaining why staff need practical scripts, supervision, and role clarity for trauma-informed responses.
- Excellent source for feasibility cautions: manual must fit restricted time, high caseloads, security routines, limited privacy, and care/control tension.
- Supports a realistic framing: correctional settings can become trauma-aware/trauma-responsive even if full TIP is structurally limited.
- Reinforces staff wellness/fidelity: trauma-informed practice cannot be sustained without staff support, reflective supervision, coaching, and organizational buy-in.

Best dissertation sections

- Problem statement: trauma-laden prison environment and risk of re-traumatization.
- Rationale: why staff-facing guidance is needed.
- Literature review: trauma-informed correctional practice: staff roles: young adults in custody.
 - *Manual theory: relationship, safety, trust, emotional regulation, "do no harm."*
- Staff training/fidelity: mandatory training, role-play, supervision, coaching, refresher needs.
- Implementation/feasibility: time/resource barriers, privacy, security constraints, culture.
- Limitations/cautions section: full TIP may be impossible without broader penal-system change.

2) Levenson & Willis (2018) – Implementing Trauma-Informed Care in Correctional Treatment and Supervision

Citation / identification

- Levenson, J. S., & Willis, G. M. (2018). "Implementing Trauma-Informed Care in Correctional Treatment and Supervision." *Journal of Aggression, Maltreatment & Trauma*.
<https://doi.org/10.1080/10926771.2018.1531959>
- Source file: doc_52028d3076f9_trauma informed in correctional settings.txt
- Stable page markers: Yes, extraction includes PAGE markers.

Purpose

- Provides a rationale for trauma-informed care (TIC) in correctional services.
- Translates SAMHSA TIC principles into practical correctional treatment/supervision behaviors.
- Introduces SHARE: safety, hope, autonomy, respect, empathy.

Sample / population

- Conceptual/practice article; no new empirical sample.
- Focuses on correctional clients, including prison, parole, probation, and mandated treatment populations.

- *Notes male correctional populations as a “last frontier” for TIC implementation (p. 3).*

Method / design

- Interdisciplinary conceptual/practice review drawing on trauma research, correctional rehabilitation, psychotherapy/common-factors literature, RNR, desistance, and SAMHSA TIC principles.
- Offers practice recommendations and case illustration.

Important facts / data points / statistics

- Correctional clients have higher rates of ACEs than the general population (p. 2).
- Early adversity can alter brain neurochemistry and compromise self-regulation/executive functioning into adulthood (pp. 2, 4).
- TIC is not trauma-specific treatment; it is a framework for safe, empowering, respectful service delivery (pp. 5, 16).
- TIC is compatible with RNR, especially responsivity: it helps clients engage with and benefit from evidence-based interventions (p. 6).
- Trauma-informed programming in women’s correctional settings has been associated with better treatment participation and reduced disciplinary infractions/conflict (p. 6).
- Correctional processes—arrest, jail, court, prison, parole/probation—can themselves be traumatic (pp. 7–10).

Key findings / arguments

- Trauma lens does not excuse crime; it improves case conceptualization and intervention strategies (pp. 2–3).
- Confrontational, risk-only, authoritarian approaches can replicate disempowering dynamics from abusive or chaotic histories and worsen resistance/dropout (pp. 3, 8, 13–14).
- Correctional settings can activate trauma responses; when stress responses are active, clients may have difficulty processing CBT/program material (p. 10).
- TIC can increase accountability by making treatment safer and more engaging, not by removing limits.
- De-escalation should validate feelings, preserve personal space, and offer behavioral choices when possible (p. 12).
- SHARE gives staff a simple practice anchor: safety, hope, autonomy, respect, empathy (pp. 13–16).

Useful direct quotes / near-quotes

- *“Viewing criminal behavior through the lens of early trauma does not excuse crime or victimization; rather, it enriches our understanding...” (p. 2).*
- *TIC creates “safe spaces for vulnerability, accountability, honesty, and ultimately, change” (p. 3).*
- *TIC is “a good example of an EBP requiring a process of critical thinking specific to each case, not a product that is packaged, tested, and delivered in a standardized fashion” (pp. 5–6).*
- *“Clients who seem combative, hostile, or resistant are often those most in need of trauma-informed responses” (p. 6).*
- *TIC is “highly compatible” with RNR and central to responsivity (p. 6).*
- *Shift paradigm from “what’s wrong with you?” to “what happened to you?” (p. 12).*
- *SHARE = “safety, hope, autonomy, respect, empathy” (p. 13).*
- *“Confrontational approaches create resistance...” (p. 14).*
- *“TIC does not replace the evidence-based cognitive-behavioral interventions... but rather it provides a strengths-based framework for delivering those interventions...” (p. 16).*

Limitations / cautions

- Conceptual/practice article rather than outcome trial.
- Acknowledges correctional safety concerns and need for clear, consistent boundaries.
- Does not provide fidelity measures or manualized intervention steps beyond principles/practice examples.

Dissertation relevance

- Core theoretical source for combining trauma-informed practice with CBT and RNR.
- SHARE can be adapted into the staff-facing manual as a mnemonic for every staff interaction.
 - *Useful for explaining why positive, respectful, autonomy-supportive interactions are not “soft”; they improve responsiveness, learning, accountability, and safety.*
- Supports using staff behavior as a delivery mechanism for correctional rehabilitation.

Best dissertation sections

- Rationale: why trauma-informed delivery matters in corrections.
- Literature review: ACEs, trauma, offending, responsiveness.
- Manual theory: SHARE, strengths-based engagement, de-escalation, corrective relational experiences.
- Staff training/fidelity: teachable practice behaviors, role-play, language shifts.
- Trauma-informed practice section: avoiding re-traumatization, safety/choice/voice.
- CBT/manual implementation: trauma-informed delivery of CBT rather than replacing CBT.
- RNR/desistance framing: responsiveness, identity change, hope, autonomy, reintegration.

3) Dumornay et al. (2022) — Improved emotion regulation following a trauma-informed CBT-based intervention associates with reduced risk for recidivism in justice-involved emerging adults

Citation / identification

- Dumornay, N. M., Finegold, K. E., Chablani, A., Elkins, L., Krouch, S., Baldwin, M., Youn, S. J., Marques, L., Ressler, K. J., & Moreland-Capua, A. (2022). “Improved emotion regulation following a trauma-informed CBT-based intervention associates with reduced risk for recidivism in justice-involved emerging adults.” **Frontiers in Psychiatry, 13**, 951429. <https://doi.org/10.3389/fpsy.2022.951429>
- Source file: doc_41c072f7a90d_EA CBT .txt
- Stable page markers: Yes, extraction includes PAGE markers.

Purpose

- Evaluates whether Roca’s trauma-informed CBT skills curriculum is associated with improvements in emotion regulation and related behavioral/life outcomes among justice-involved emerging adult men.

Sample / population

- N = 344 emerging adult men ages 17–24 engaged in Roca’s intervention model.
- Sites: four Massachusetts sites and one Baltimore, Maryland site.
- Original dataset: 1,896 participants; analysis limited to those with at least three CBT-related encounters/data points across a three-year window.

- Demographics/risk factors at intake:
 - 55.8% Hispanic/Latino.
 - 82.8% non-white.
 - 63.7% dropped out of high school without GED.
 - 68.0% not employed in prior 6 months.
 - 77.3% currently justice-system involved.
 - 73.8% drug involved.
 - 58.7% street/gang involved.

Method / design

- Retrospective program evaluation using deidentified routine program data; IRB exempt.
- Naturalistic, nonrandomized design; no true control group.
- Roca intervention: trauma-informed, CBT-based, peer-delivered model with transformational relationships, relentless outreach, life skills, job readiness, healthy habits, parenting, stage-based individualized programming, and partnerships with police/probation.
- CBT delivery: paraprofessional youth workers trained by master's-level clinicians; flexible, brief CBT skill delivery in safe/community settings.
- CBT sessions could range from 10–60 minutes, with frequency ranging from monthly to yearly over a three-year window depending on participant engagement.
- Measures: DERS/DERS-SF emotion regulation, Life Distress Inventory, Relationship Interactions Questionnaire, Drug/Alcohol Use Questionnaire, Criminal Thinking Scale.
- Analyses: repeated-measures ANOVA, paired t-test, Pearson correlations, ANOVAs; Bonferroni correction for LDI item analyses.

Important facts / data points / statistics

- Justice-involved male youth have high trauma/mental health needs: approximately 66% of incarcerated young men meet criteria for at least one mental health disorder; an estimated 90% of justice-involved youth have experienced at least one traumatic event (pp. 1–2).
- Emerging adulthood/transition age youth: ages 16–25 are particularly vulnerable and have high mental health problem rates (p. 2).
- Roca serves emerging adult men ages 17–24 who are high risk and often not ready/willing/able to engage traditional services (pp. 2, 4).
- Employment distress improved over time: $F(2,319) = 8.5, p < .001, \eta^2 = .026$ (p. 6).
- Education distress improved over time: $F(2,320) = 10.7, p < .001, \eta^2 = .032$ (p. 6).
- Drug-problem distress reduced: $F(2,286) = 304.3, p < .001, \eta^2 = .516$; importance of drug treatment felt less urgent: $F(2,287) = 165.9, p < .001, \eta^2 = .366$ (p. 6).
- Alcohol consumption showed marginal increase: $F(2,295) = 3.1, p = .045, \eta^2 = .010$ (p. 6).
- Whole-sample emotion regulation did not significantly improve on DERS-SF, $p = .256$ (p. 6).
- Approximately half improved in emotion regulation: $N = 124, 46\%$; $N = 146, 54\%$ did not improve (p. 7).
- Improved emotion regulation group had lower overall life distress at final visit: $F(1,269) = 9.6, p = .002, \omega^2 = .031$ (p. 7).
- Better emotion regulation correlated with less life distress: $r = .248, p < .001, N = 310$ (p. 7; note direction reflects scoring where lower DERS = better regulation).
- Improved emotion regulation was associated with lower distress in 10 of 18 life domains; after correction, sexual behavior and religion survived categorical analyses; sex, recreation/leisure, social life, and religion survived

continuous analyses (pp. 7–8).

- Emotional awareness improved significantly: $t(310) = -3.08$, $p = .002$, Cohen's $d = .174$ (p. 8).
- Improved impulse control correlated with decreased life distress, improved relationship functioning, decreased criminal thinking, and decreased alcohol use; p 's $< .05$, r 's = .125 to .159 (p. 9).

Key findings

- Roca participation was associated with decreased employment and education distress across the whole sample.
- Overall emotion regulation did not significantly improve for the full sample, but participants who did improve in emotion regulation showed better life outcomes.
- Specific emotion regulation domains—emotional awareness, clarity, goal-directed behavior, impulse control—were linked to improved outcomes.
- Peer-delivered, flexible, trauma-informed CBT may be feasible for high-risk emerging adult men who often do not engage traditional services.
- Trust and safety may be prerequisites for behavior/emotion change and may take time with participants exposed to developmental trauma.

Useful direct quotes / near-quotes

- *Roca engages participants who “otherwise would not be eligible or able to participate in other youth driven services” because of high risk for violence/reincarceration (p. 2).*
- *Roca’s model centers “meeting individuals where they are” (p. 2).*
- *Trauma can manifest as “impulsivity, inability to cope with intense emotion, poor decision-making, high novelty seeking behavior, and risk-taking” (p. 3).*
- *“Improving or strengthening emotion regulation skills may be an important strategy for reducing justice-involved youths’ risk for (re)incarceration and improving their life outcomes” (p. 3).*
- *Roca’s trauma-informed model adheres to “safety... prevention of re-traumatization, collaboration, empowerment, mutuality, voice and choice, and respect for shared expertise” (p. 4).*
- *Peer specialists “literally” meet participants where they are—in homes or on the street—to increase engagement (p. 4).*
- *“Behavior and emotion change take time to develop” and trust/safety is often a prerequisite (p. 9).*
- *Study provides “initial evidence that a community-based, CBT intervention for justice-involved emerging adult men can positively impact affective, behavioral, and life outcomes...” (p. 10).*

Limitations / cautions

- Naturalistic program evaluation; no random assignment or true control group.
- Only participants with at least three CBT encounters included; many original participants did not meet engagement threshold.
- No standardized dose analysis; participants received varied amounts/frequencies of CBT.
- Did not fully account for environmental/situational contexts or other supports.
- Possible youth worker/training/supervision differences not analyzed.
- Generalizability limited to emerging adult males ages 17–24 and one organization’s program.
- Need objective reincarceration/recidivism outcomes in future research.

Dissertation relevance

- Highly aligned with Destyne’s target population: justice-involved emerging adult men.

- Provides empirical support for trauma-informed CBT skill-building, peer/paraprofessional delivery, flexible brief sessions, and emotion regulation as a mechanism related to recidivism risk factors.
- Supports manual modules on emotional awareness, emotion labeling, impulse control, goal-directed behavior, relationship skills, and substance-use coping.
- Reinforces that staff/paraprofessionals can deliver basic CBT skills with training/coaching.
- Important caution: implementation needs dosage tracking, fidelity monitoring, engagement strategies, and realistic expectations.

Best dissertation sections

- Problem statement: trauma and mental health needs among justice-involved young men.
- Rationale: need for accessible, flexible interventions for high-risk emerging adults.
- Literature review: emerging adulthood, trauma, emotion regulation, CBT, recidivism risk.
- Manual theory: CBT + emotion regulation + trauma-informed engagement.
- CBT/emerging adults section: exact population match.
- Staff training/fidelity: peer/paraprofessional delivery, coaching, flexible delivery, dose tracking.
- Implementation/feasibility: brief skills, outreach, meeting clients where they are, naturalistic barriers.

4) Taxman (2014) — Building effective service delivery mechanisms for justice-involved individuals: an under-researched area

Citation / identification

- *Taxman, F. S. (2014). "Building effective service delivery mechanisms for justice-involved individuals: an under-researched area." *Health & Justice, 2*, 2.*
- Source file: doc_512de71a206b_service delivery .txt
- Stable page markers: Yes, extraction includes PAGE markers.

Purpose

- *Argues that justice/health research must move beyond "what program works?" to study how services are delivered, by whom, under what conditions, with what fidelity, and with what system supports.*
- Reviews service delivery, program fidelity, implementation, engagement, geographic access, and health-justice process issues.

Sample / population

- Short report/conceptual review; no new empirical sample.
- Focuses broadly on justice-involved individuals and systems that serve them.

Method / design

- Narrative/conceptual review of health-justice service delivery literature, fidelity research, implementation science, process measures, spatial access, and service-system frameworks.

Important facts / data points / statistics

- Justice-involved populations have high rates of infectious disease, behavioral health disorders, chronic disease, and comorbid needs (p. 1).

- Fidelity includes adherence to treatment approach/orientation, use of a manual, prescribed dosage/frequency, and delivery consistent with research basis (p. 2).
- Program integrity factors from mental health include components/orientation, dosage, staff certification/training, program management, quality assurance/control, evaluator involvement, benchmarks (p. 2).
- Andrews & Dowden review of 273 studies: larger effect sizes for programs with integrity, specific treatment models, staff with good interpersonal skills trained in the program, clinical supervision, and evaluator involvement (p. 2).
- Lipsey & Landenberger: CBT programming, evaluator presence, and more treatment sessions positively impacted client outcomes (p. 2).
- Lipsey juvenile justice meta-analysis: larger effects when programs included counseling, client skill-building, and interventions based on family therapies or CBT (p. 2).
- NIDA principles include: treatment readily available; attends multiple needs; flexible; adequate time in treatment; counseling/behavioral therapy; integrated care for coexisting disorders; long-term recovery process (pp. 3).
- Pearson et al. meta-analysis of 232 studies: differential effects for strategies including contingency management (.13-.29), CBT (.02-.20), therapeutic community (.18-.53); lower-quality studies and publication bias had larger effect sizes (p. 3).
- NIC effective correctional management principles include actuarial risk/needs assessment, enhancing intrinsic motivation, targeting interventions by risk/need/dosage, staff skills, rewards, community support, process/practice measurement, and feedback (p. 3).
- Treatment initiation/engagement process measures: initiation within 14 days after identification/assessment; engagement = two treatment services within 30 days after initiation (pp. 6-7).
- Clients who initiated and engaged in care had reduced rearrest and reincarceration compared with those who initiated but did not participate (Garnick et al., p. 7).
- Reducing waitlists and accelerating treatment initiation can improve treatment retention (p. 7).
- Geographic access matters: in one California parolee study, likelihood of recidivism reduced by 41% when parolees lived within two miles of any service provider (p. 9).

Key findings / arguments

- Good program design alone is insufficient; outcomes depend on implementation, fidelity, staffing, supervision, dosage, quality assurance, and organizational support.
- Many evidence-based practice barriers are staff/organizational barriers: officer attitudes, lack of trust in assessment, preference for personal judgment, paperwork-centered processes, authoritarian vs. shared decision-making, lack of client involvement, resistance to treatment, punitive/control environments.
- Facilitators include coaching, addressing staff concerns in training, value clarification, motivational interviewing, rapport-building early, tailoring case plans to risk/needs assessment, performance measures, leadership support, line-staff involvement, rewards, and feedback loops.
 - *Service process matters: "if you build it, they will come" is not true; programs may fail due to low referral, enrollment, engagement, or retention.*
- Justice involvement should be treated partly as a public health/service-access issue because criminal risk behaviors and unmet health/social needs co-occur.

Useful direct quotes / near-quotes

- *Traditional efficacy studies do not answer "who delivers care, how care is delivered, in what settings, under what conditions... and how to improve care" (p. 1).*
- *"A good design may not be enough to guarantee good program outcomes" (p. 2).*
- *Fidelity includes "the use of a manual," "prescribed dosage and frequency of sessions," and delivery similar to the research basis (p. 2).*

- The design-implementation-fidelity paradox requires understanding factors needed to produce positive outcomes (p. 2).
 - *"It is not always the case that 'if you build it, they will come'" (p. 6).*
- Health disparities and disenfranchisement can interfere with justice-involved individuals pursuing services; service processes must be positive and address social barriers (p. 8).
 - *"Treating criminal offending as a public health factor advances the notion that offending can be adjusted through interventions and services..." (p. 10).*

Limitations / cautions

- Short report/narrative review; not a direct intervention evaluation.
- Broad health-justice focus; not specific to emerging adult men or prison manuals.
- Some cited implementation principles lack meta-analytic testing.

Dissertation relevance

- Essential for manual feasibility/fidelity chapter.
- Supports including fidelity checklists, session dosage, staff training, coaching, quality assurance, performance feedback, and engagement/retention metrics.
- Useful for explaining that a staff-facing manual must specify not just content, but delivery mechanisms.
- Supports staff buy-in and organizational culture planning.
- Highlights geographic/service-continuity issues relevant for reentry or community supervision adaptations.

Best dissertation sections

- Problem statement/rationale: unmet needs and system delivery gaps.
- Implementation/feasibility: how programs fail or succeed in real correctional systems.
- Staff training/fidelity: manual use, dosage, certification/training, supervision, QA, evaluator, benchmarks.
- Program design chapter: logic model, service process measures, referral/engagement/retention.
- RNR/manual alignment: assessment, risk/need/dosage matching, intrinsic motivation, rewards.
- Evaluation plan: process measures, fidelity ratings, outcome tracking, feedback loops.

5) Klapow, Shea, Bohn, & Hoffman — The Effectiveness of Positive Psychology-Based Interventions in Prisons on Well-Being: a Systematic Review and Meta-Analysis

Citation / identification

- Klapow, M. C., Shea, K., Bohn, T., & Hoffman, D. *"The Effectiveness of Positive Psychology-Based Interventions in Prisons on Well-Being: a Systematic Review and Meta-Analysis."* Manuscript text provided.
- Source file: doc_515899fbf565_PWB Manuscript_FINAL.txt
- Stable page markers: No stable PAGE markers in extraction; DOCX-style text only.

Purpose

- Systematically reviews and meta-analyzes positive psychology-based interventions (PPIs) in prison settings and their effects on psychological well-being.
- Aims to characterize prison PPIs and preliminarily evaluate effectiveness.

Sample / population

- 9 studies, 662 incarcerated adult participants.
- 428 participants in PPI groups; 234 in control groups.
- 4 RCTs are stated in abstract; results section describes five randomized controlled studies and four non-controlled studies.
- Countries: USA (1), China (5), Iran (1), Indonesia (1), Australia (1).
- Gender: 5 studies with male offenders; 4 with female offenders.
- Ages ranged 18-61.

Method / design

- Systematic review and meta-analysis conducted according to PRISMA guidelines.
- Search conducted March 2024 across 8 databases: PsycInfo, MEDLINE, CINAHL, PubMed, Web of Science, SCOPUS, EMBASE, ProQuest.
- Inclusion: incarcerated adults over 18; group-based psychoeducational interventions targeting positive psychological constructs such as well-being, strengths, self-efficacy, hope.
- Excluded: psychotic disorders, externalizing disorders, complex mental health problems; psychotherapy; crisis management; episodic risk behaviors.
- Risk of bias assessed using ROB2 and ROBINS-I.
- Random-effects meta-analysis in R; Hedge's g used due to small sample size.

Important facts / data points / statistics

- Search yielded 6,334 studies; 3,950 after deduplication; 9 included.
- Included studies published 2015-2021.
- Interventions:
 - 4 single-component PPIs: gratitude, hope, kindness.
 - 5 multicomponent programs: PERMA/character strengths, resilience, mindfulness, CBT + positive psychology, well-being/self-acceptance/self-esteem/positive relationships.
 - Duration: 5 weeks to 8 months; 8 studies lasted 4-12 weeks.
- Outcomes: 31 different measurement scales; 8 studies measured well-being; 3 also measured distress/mental health symptoms; 1 measured depression only.
- Controlled studies: large significant between-group effect on well-being, Hedge's $g = 0.76$, $p < .0001$, $n = 469$; 95% CI = 0.56-0.96; $Z = 7.53$.
- Within-group effects: large significant effect, Hedge's $g = 0.66$, $p < .0001$, $n = 180$; 95% CI = 0.44-0.87; $Z = 5.98$.
- Heterogeneity: between-group $I^2 = 0.0$, $Q = 1.67$, $df = 4$, $p = .80$; within-group $I^2 = 0.0$, $Q = 0.17$, $df = 3$, $p = .98$; authors caution this may reflect small number of studies/large standard error.
- Publication bias: Egger's test nonsignificant for controlled studies, $t = 2.176$, $p = .118$; uncontrolled studies, $t = 1.66$, $p = .971$; small sample limits assessment.
- Risk of bias: all 5 randomized studies rated high overall risk of bias; nonrandomized studies: 2 serious, 2 moderate overall risk.

Key findings

- PPIs appear to significantly improve psychological well-being in prison settings, especially subjective well-being and life satisfaction.
- Evidence is promising but low certainty due to small number of studies, poor methodological quality, heterogeneous interventions, and high risk of bias.
- PPIs may be scalable because they can be lay-delivered or self-administered and used as non-specialized support for general prison populations.
- PPIs may fit a stepped-care psychosocial support model in prisons and complement specialized services.
- PPIs align with strengths-based correctional rehabilitation and Good Lives Model principles.

Useful direct quotes / near-quotes

• Abstract: “9 studies (4 RCTs) were included, representing 662 participants.”
• Abstract: “large, significant effects on psychological well-being in comparison with control arms (Hedge’s $g = 0.76$).”
• Abstract: “The overall quality of included studies was poor... limiting the certainty of these findings.”
• PPIs include “identifying and expressing positive emotions, discovering strengths, practicing flow, promoting optimism, hope, self-compassion, and gratitude, completing acts of kindness, and practicing mindfulness.”
• “Prison populations... present a unique context for study” because they are less educated, poorer, overrepresented by ethnic minorities, and have less access to resources.
• Discussion: PPIs and similar strengths-based programs may improve “self-efficacy, confidence, and optimism,” constructs associated with engagement in other programs.
• Discussion: PPIs are “easily administered by lay people,” can be self-administered, and may be appropriate for populations with and without mental health problems.
• Conclusion: “There is evidence to support the effectiveness of positive psychology-based interventions in improving psychological well-being in prison settings, though with limited certainty.”

Limitations / cautions

- Only 9 studies; generalizability limited.
- High risk of bias across all randomized studies; serious/moderate bias in nonrandomized studies.
- Inconsistent outcome measurement and reporting.
- Heterogeneous intervention content/duration; some included CBT elements, obscuring PPI-specific effects.
- Small samples may overestimate effects.
- No stable page markers in extracted DOCX text.

Dissertation relevance

- Main support for positive psychology component of Destynee’s manual.
- Justifies using strengths, hope, gratitude, self-efficacy, optimism, mindfulness, and positive relationships in prison programs.
- Supports a low-intensity, staff-deliverable or paraprofessional-deliverable model.
- Must be framed carefully: promising but not definitive; manual should include evaluation and fidelity because the evidence base is still weak.
- Connects well with Good Lives Model/desistance and with engagement-enhancing goals.

Best dissertation sections

- Rationale: why well-being matters in correctional rehabilitation.

- Positive psychology section: PPIs in prison, well-being outcomes, strengths-based rehabilitation.
- Manual theory: PERMA/strengths/hope/gratitude/self-efficacy as protective and engagement-enhancing constructs.
- Implementation/feasibility: lay delivery, brief psychoeducation, stepped-care fit.
- Limitations/evaluation plan: need rigorous trials, fidelity, clearer reporting.
- Desistance/Good Lives section: strengths-based identity, hope, positive future orientation.

Cross-Article Themes

1) Staff delivery is not a side issue; it is the intervention pathway.

- Vaswani & Paul show staff interactions can either support or undermine trauma-informed practice.
- Levenson & Willis argue that relational modeling, respect, empathy, and autonomy are core mechanisms of TIC.
- Taxman shows program quality depends on staff training, interpersonal skills, supervision, fidelity, dosage, and feedback.
- Dumornay et al. show paraprofessional/peer youth workers can deliver CBT skills when trained/coached.
- Klapow et al. suggest PPIs may be feasible because they can be lay-delivered/self-administered.

2) Trauma-informed practice should improve responsiveness, not replace accountability.

- Levenson & Willis explicitly connect TIC to RNR responsiveness.
- Dumornay et al. show emotion regulation is linked to life distress, criminal thinking, relationships, and substance/alcohol outcomes.
- Vaswani & Paul caution that prison's care/control tension limits full TIP implementation.
- A dissertation manual can frame TIP as a delivery style that helps participants learn and use CBT/positive psychology skills.

3) Emerging adult men need developmentally responsive skills, especially emotion regulation.

- Dumornay et al. directly focus on 17-24-year-old men and link trauma, brain development, emotion dysregulation, and recidivism risk.
- Vaswani & Paul emphasize young people in custody are at a critical developmental stage.
- Levenson & Willis support trauma lens for aggression, impulsivity, self-regulation, and executive functioning.

4) Feasibility/fidelity must be designed from the beginning.

- Taxman is the strongest source for fidelity: manualization, dosage, staff training, supervision, QA, evaluator involvement, benchmarks, and feedback.
- Vaswani & Paul show real implementation barriers: time, privacy, waitlists, staff emotional labor, inconsistent culture.
- Dumornay et al. show flexible delivery is feasible but dose/engagement variability limits interpretation.
- Klapow et al. show promising effects but weak evidence quality; Destynee's manual should include evaluation tools.

5) Positive psychology and CBT are compatible with trauma-informed corrections.

- Klapow et al. support PPIs for prison well-being and scalable strengths-based programming.
- Dumornay et al. support CBT-based emotion regulation skills for justice-involved emerging adult men.
- Levenson & Willis explain TIC can improve delivery of CBT rather than replace it.

- Taxman supports CBT and skill-building as evidence-based components when delivered with fidelity.

Paste-Ready Synthesis Paragraphs

Synthesis paragraph 1 — Rationale/problem statement

Justice-involved and incarcerated emerging adult men often present with layered trauma histories, unmet behavioral health needs, and developmental vulnerabilities that complicate traditional correctional programming. Dumornay et al. (2022) note that approximately 66% of incarcerated young men meet criteria for at least one mental health disorder and an estimated 90% of justice-involved youth have experienced at least one traumatic event. Vaswani and Paul (2019) similarly describe prison as a trauma-laden environment where prior adversity intersects with environmental stressors such as fear, violence, isolation, noise, lack of privacy, and rigid control. Together, these sources support the need for rehabilitation programming that does not merely target risk, but also addresses trauma-related responsivity barriers such as mistrust, emotion dysregulation, impulsivity, shame, and difficulty engaging with authority figures.

Synthesis paragraph 2 — Manual theory/intervention model

A staff-facing rehabilitation manual can integrate trauma-informed practice, basic CBT, and positive psychology by treating staff interactions as a primary mechanism of responsivity and skill reinforcement. Levenson and Willis (2018) argue that trauma-informed care is compatible with RNR because it strengthens the responsivity conditions needed for clients to learn and apply evidence-based interventions; their SHARE model—safety, hope, autonomy, respect, and empathy—offers a practical staff framework. Dumornay et al. (2022) provide population-specific support for trauma-informed CBT with justice-involved emerging adult men, finding that improvements in emotion regulation were associated with lower life distress and improvements in domains related to relationships, criminal thinking, and substance/alcohol outcomes. Klapow et al. further support adding positive psychology components, reporting significant improvements in prison well-being following PPIs, while cautioning that the evidence remains limited by methodological weaknesses.

Synthesis paragraph 3 — Implementation/fidelity/feasibility

The literature also indicates that correctional rehabilitation manuals must be designed for implementation conditions, not ideal clinical settings. Taxman (2014) emphasizes that program outcomes depend on service delivery mechanisms such as manualization, dosage, staff training, clinical supervision, quality assurance, evaluator involvement, process measures, and feedback. Vaswani and Paul (2019) show why these supports are critical in prison contexts: staff may be willing to respond to trauma but face limited time, high need, insufficient privacy, hypermasculine culture, care/control tensions, and lack of supervision for emotionally demanding work. Therefore, Destynée's manual should include brief skill modules, concrete staff scripts, fidelity checklists, realistic dosage expectations, refresher training, coaching/supervision structures, and process measures for engagement and completion.